STAKEHOLDER REPORT

Lumen: helping people prepare for clearer care-team conversations

A cautious, source-grounded prototype for explaining cancer pathology-report wording — designed to show where every statement came from, keep quiet when evidence is missing, and never pretend to be a clinician.

Audience: product, clinical advisors, partners, and demo reviewers · **Tone:** disclosure-first · **Scope:** English, text-only, synthetic prototype

Purpose

Turn unfamiliar report wording into a careful briefing and neutral questions for the appointment.

Stance

Evidence before eloquence. An untraceable answer is a failure. Absence is not a negative result.

Status

Software prototype with automated checks. Not clinically validated. Not for patient use.

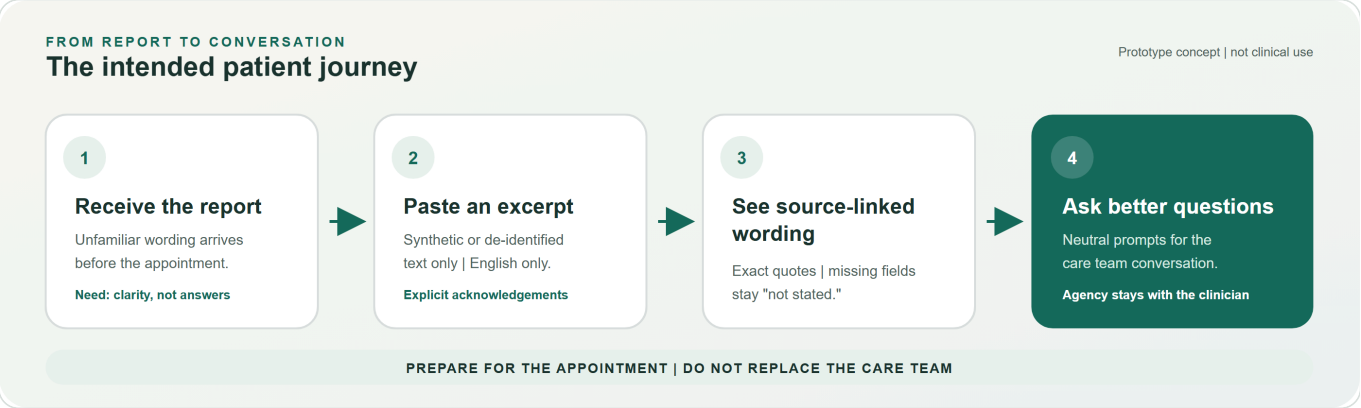


Figure 1. Intended experience: excerpt → evidence checks → exact quotes → care-team questions.

Non-negotiable disclosure: Lumen is a synthetic-only, English-only software prototype — not medical advice, not a diagnostic product, and not clinically validated. Confirm every meaning with a care team.

The human problem

People may see pathology wording through portals or on paper before a clinician has explained it. Reports use specialist language. Access alone does not create understanding. The result can be confusion, unnecessary worry, or a quieter appointment than the person hoped for.

> **“Help me understand what this document says, show me where each explanation came from, and help me ask useful questions — without telling me what my diagnosis or treatment should be.”**

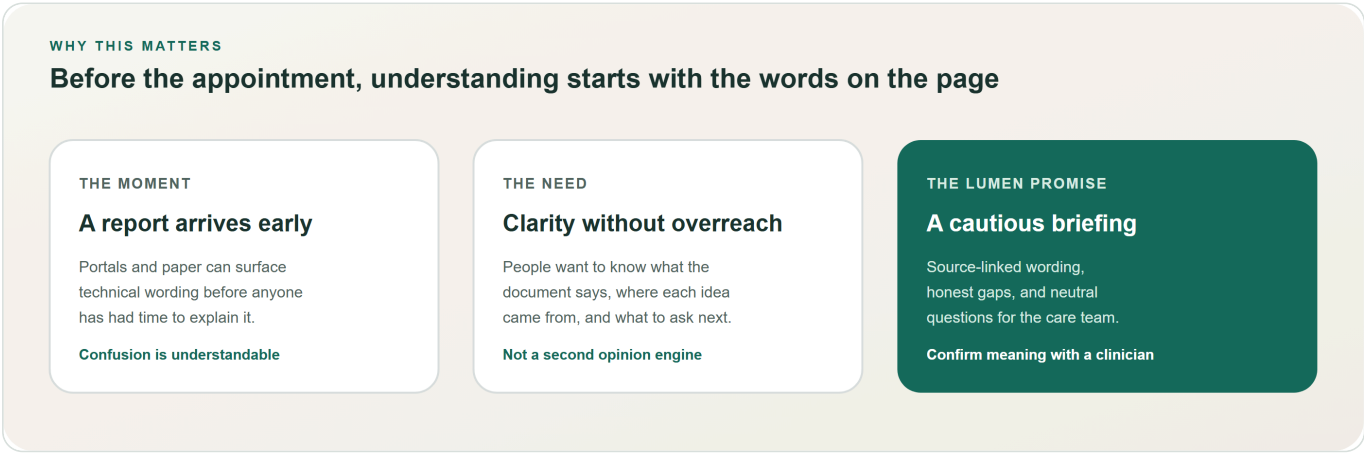


Figure 2. From early report access to a carefully bounded briefing.

Who Lumen is for

Intended

Adults preparing for a care-team appointment after a cancer-related report; caregivers supporting with consent; people who need clearer wording and better questions.

Not intended

Children, emergency care, clinical decision-making, unsupervised treatment interpretation, or any use of identifiable patient information in this prototype.

What “good” looks like

- A person can inspect exact source wording for each shown statement.
- Missing facts remain visibly “not stated,” not silently filled in.
- Out-of-scope requests receive clear, fixed refusals.
- The appointment still belongs to the clinician — Lumen only helps prepare.

Design principles

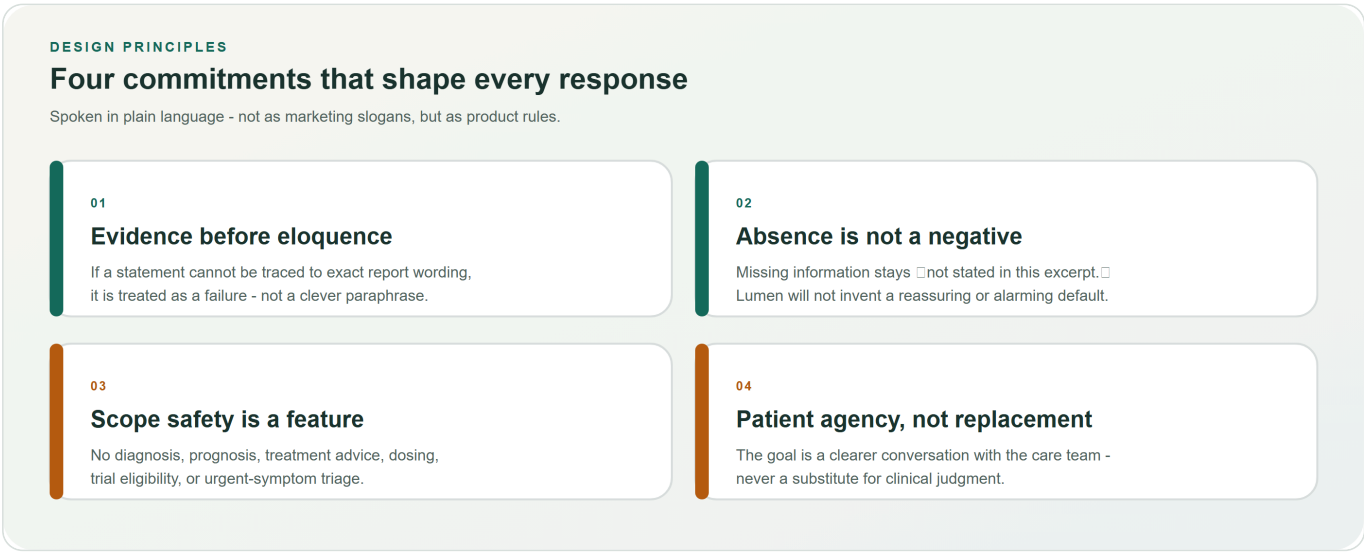


Figure 3. Four commitments that govern product behavior.

Scope safety is a feature

Lumen’s value is as much about what it refuses as what it explains. The product stays useful by staying narrow.



Figure 4. Inside the boundary versus outside the boundary.

Operating language: Lumen explains report wording; it is not medical advice. It does not diagnose, predict outcomes, or recommend treatment. Confirm meaning with your care team.

A simple information journey

In everyday terms: a person submits an excerpt with explicit acknowledgements; the service applies evidence and safety checks; the interface either shows source-linked wording and questions, or withholds claims and explains why.

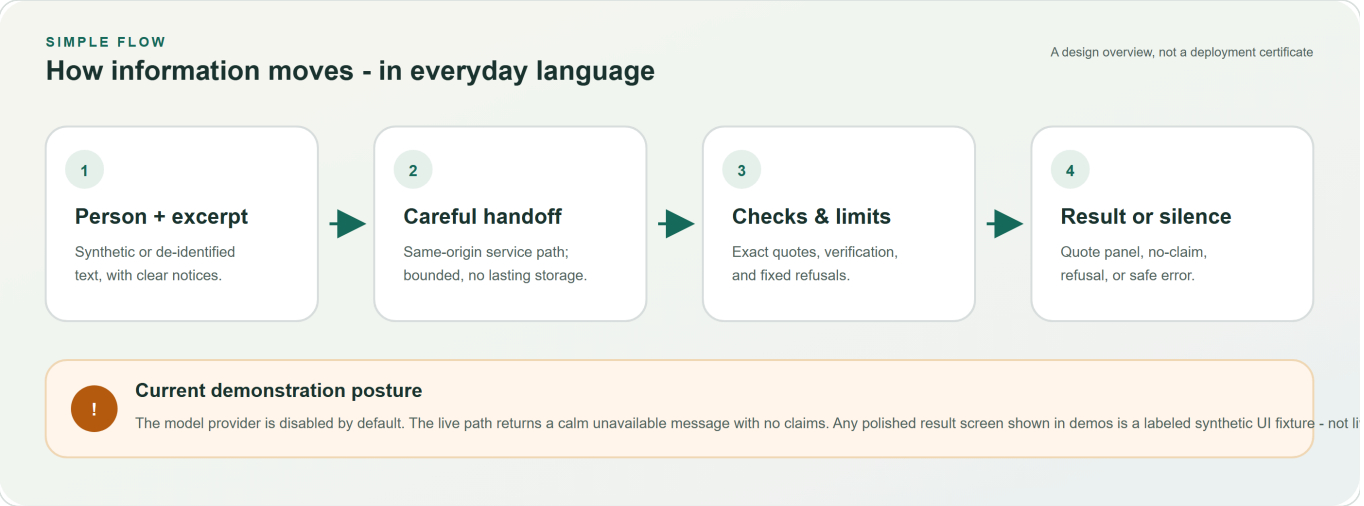


Figure 5. High-level flow with the current demo disclosure.

The evidence gate

Every statement that appears must earn its place. Exact source wording, verification, and safety limits decide whether content is shown, withheld, or refused.

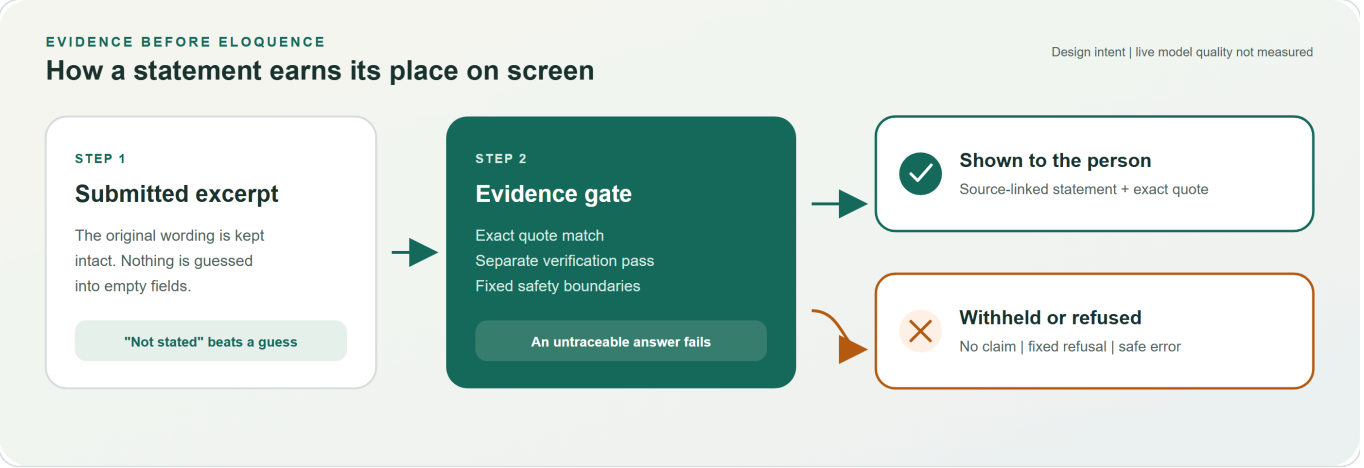


Figure 6. Supported path versus withheld or refused outcomes.

If this happens…	Lumen's response
Wording is present and supported	Show a plain-language statement with an exact quote control
A field is missing from the excerpt	Mark it "not stated" — do not invent a value
Request asks for prognosis or treatment advice	Return a fixed refusal and point back to the care team
Evidence cannot be verified	Fail closed: no unsupported claims are displayed

What has been verified — carefully phrased

The figures below describe software discipline and fixture integrity. They are not model accuracy scores and not clinical outcome results.

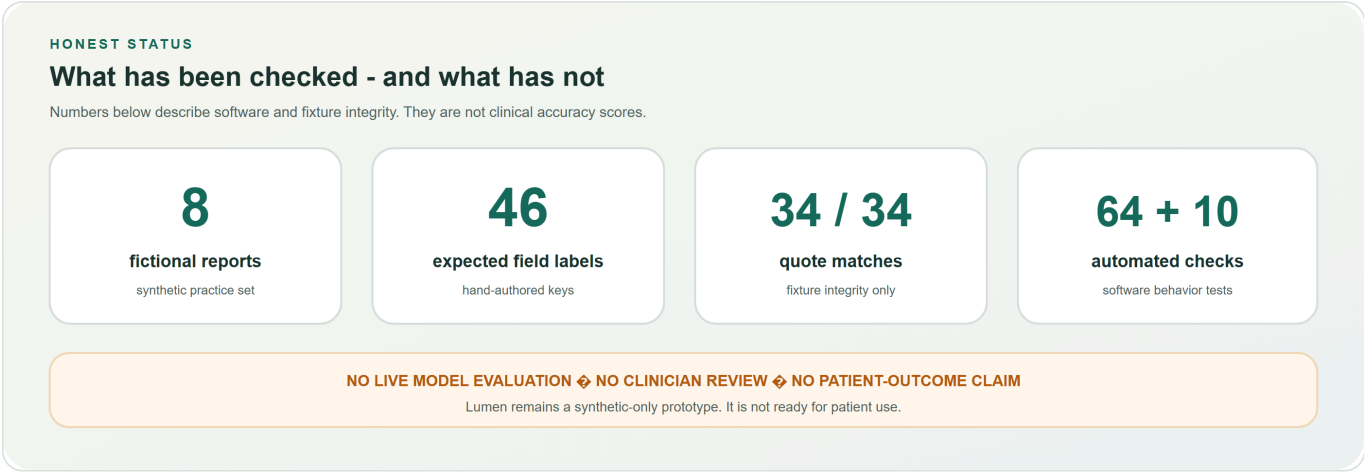


Figure 7. Honest status snapshot for stakeholder review.

Present strengths

Clear product boundary; source-linked design; fixed refusals; fail-closed posture; synthetic demo discipline; automated regression coverage for key behaviors.

Not yet claimed

Live model quality; clinician-labeled evaluation; patient-outcome benefit; multi-language support; PDF/image intake; deployment privacy certification.

Recommended next steps

1. Independent clinician and patient review of task framing, safety language, and examples.
2. Human-labeled evaluation protocol before any accuracy narrative is used externally.
3. Privacy, security, accessibility, and deployment review if a controlled pilot is ever considered.
4. Keep every public demo disclosure-first: synthetic data, provider status, and “not medical advice.”

Closing line for stakeholders: Lumen aims to make pathology wording discussable — not decidable. The care team remains the authority. Until clinical validation exists, treat Lumen as a carefully bounded software prototype only.

Contact posture for demos: Use synthetic excerpts only. Label fixture screens as synthetic UI demos. Do not present disabled-provider failures as successful model answers.